

9. REVENUE STREAM

Two revenue engines, sequenced by evidence. Olera's revenue model has two independent but reinforcing engines. CareNavigator remains free to families and basic family-provider connections remain free to providers. The near-term engine is Caregiver Staffing: providers pay when Olera helps them successfully hire workers, whether the staffing need arises from turnover, existing clients, externally generated referrals, growth, or a capacity constraint identified through CareNavigator. The emerging engine is institutional CareNavigator contracting with organizations responsible for populations of older adults, including Medicare Advantage plans, accountable care organizations, Medicaid organizations, and health systems. Staffing monetizes successful workforce recruitment; institutional contracts monetize the navigation, execution, and outcomes infrastructure as CRP evidence establishes its value.

Caregiver Staffing: revenue follows successful hires. The Staffing model is intentionally simple: successful hires, times realized revenue per successful hire, times active county markets. CRP Year 1 is an engineering year and generates no Staffing revenue. In Year 2, Olera deploys Staffing free to providers so the project can establish applicant acquisition, provider hiring, placement, retention, and market-to-market reproducibility before price is introduced. Paid testing begins in Year 3. The working base-case price is \$250 per successful hire, with \$150 and \$350 as sensitivity bounds. This is an Aim 3 pricing hypothesis, not an asserted market price. It is economically plausible relative to the burden providers already bear: home-care benchmarking reports approximately 75% professional-caregiver turnover, while industry estimates place recruiting and training cost at up to approximately \$2,700 per replacement.¹⁻³

Model input	Base projection	Range or benchmark	Basis
Staffing equation	counties × hires/mo × paid months × \$/hire		Derived arithmetic
Successful hires per county per month	10	Expected mature range 10 to 30	Conservative Olera and CRP hypothesis
Price per successful hire	\$250	\$150 to \$350 sensitivity	Aim 3 pricing hypothesis
Home-care turnover		About 75%	Published benchmark ^{1,3}
Recruiting and training burden		Up to about \$2,700 per replacement	Published benchmark ²
Institutional equation	active contracts × annual contract value		Derived arithmetic
First institutional revenue	Post-CRP Year 4		Planning assumption
Institutional relationships	About 3 by Year 5		Planning assumption
Annual value per relationship	About \$250K	Negotiated after evidence	Planning hypothesis, not a benchmark
Institutional payment precedent		CMS GUIDE; Medicare ACO market	Published buyer and economic precedent ⁴⁻⁶

Table 6. The projection separates published benchmarks from Olera planning assumptions and CRP hypotheses.

For the financial projection, Olera holds every paid market at only 10 successful hires per month. At \$250 per hire, that equals \$30,000 annualized Staffing revenue per county. Olera expects mature markets may support approximately 10 to 30 successful hires per month, but the model does not require that maturation. Across eight CRP markets, the conservative case produces a \$240,000 annualized Staffing run rate; the same markets would produce \$480,000 at 20 hires per month and \$720,000 at 30. Within-market maturation is therefore upside that the CRP measures rather than revenue the forecast presumes.

THE UNIT, HELD CONSTANT

\$30,000

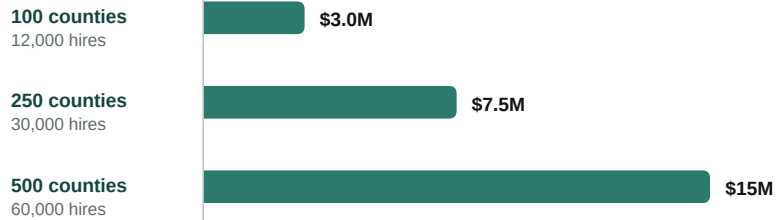
a year, from one county
10 successful hires a month at \$250 each

Eight CRP markets \$240,000 a year

Excluded from the forecast

at 20 hires a month \$480,000
at 30 hires a month \$720,000

POST-CRP REPLICATION, SAME 10 HIRES A MONTH



annual Caregiver Staffing revenue, institutional revenue excluded

Figure 11. Replication economics at the deliberately conservative 10-hire projection. Mature-market throughput above 10 hires a month and all institutional CareNavigator revenue are excluded.

The five-year model is an early commercialization case, not the scale ceiling. At the same conservative assumptions, 100 active counties produce approximately 12,000 successful hires and \$3.0 million in annual Staffing revenue; 250 counties, 30,000 hires and \$7.5 million; and 500 counties, 60,000 hires and \$15 million. At 500 counties, 60,000 successful provider hires are equivalent in scale to approximately 8% of the roughly 760,500 annual U.S. home health and personal care aide openings projected by BLS.⁷ Successful hires are not assumed to equal unique new workforce entrants: the CRP will separately measure unique workers recruited, prior workforce status, repeat placements, retention, and resulting provider capacity. Health-profession applicants are the initial recruitment wedge; the workforce-entry infrastructure is designed to expand to broader labor populations as the model scales.

Institutional CareNavigator: contracts follow outcomes evidence. The institutional engine is modeled separately and more conservatively. Olera assumes no institutional revenue during the CRP. Years 1 to 3 instead test the intermediate outcome on which the institutional value proposition depends: whether recognized needs progress through navigation, funding, execution, and ultimately established care; why pathways fail when they do not; and what happens longitudinally after care is or is not established. This matters economically because unmet home- and community-based service needs have been associated with substantially greater acute-care utilization: an AHRQ evidence map summarizes a 13-state Medicaid HCBS study in which participants reporting unmet needs had greater emergency department use (52% versus 34%) and hospital or rehabilitation stays (36% versus 24%).⁸ The CRP does not assume that CareNavigator prevents these downstream events; it generates the care-establishment and longitudinal evidence needed to determine whether that value proposition is real.



There is already precedent for organizations responsible for health outcomes to pay for care-management and coordination infrastructure: CMS's GUIDE Model uses per-patient-per-month dementia care-management payments for coordination and caregiver support, and in 2026 the Medicare Shared Savings Program includes 511 ACOs serving 12.6 million Traditional Medicare beneficiaries.⁴⁻⁶ These sources establish the buyer class and purchasing logic; they do not establish Olera's future price. Accordingly, the five-year model treats institutional revenue as evidence-gated contracts rather than multiplying an unvalidated PMPM across a hypothetical health plan. The base case assumes the first paid relationship in post-CRP Year 4 and approximately three active relationships in Year 5. A working \$250,000 annual value per mature relationship is used only as a planning hypothesis and will be replaced by negotiated pricing.

	CRP Y1	CRP Y2	CRP Y3	Post-CRP Y4	Post-CRP Y5–Y10 potential
Commercial stage	Build	Validate free	Monetize	Expand	Scale to national replication
Staffing markets	0	~8 free	~8 paid	~15	~25 to 500 counties
Staffing revenue	\$0	\$0	~\$120K*	~\$450K	~\$0.75M–\$15.0M/yr
Institutional relationships	0	0	0	~1	~3 to 10+
Institutional revenue	\$0	\$0	\$0	~\$150K	~\$0.75M–\$2.5M+/yr
Total commercial revenue	\$0	\$0	~\$120K	~\$600K	~\$1.5M–\$17.5M+/yr

Table 7. Illustrative base case through post-CRP Year 4. The final column widens Year 5 into a Year 5 to Year 10 replication range whose lower bound is the Year 5 base case charted in Figure 12. *Year 3 assumes approximately six paid-month equivalents across the eight CRP markets; the resulting exit Staffing run rate is approximately \$240K a year at the same conservative throughput.

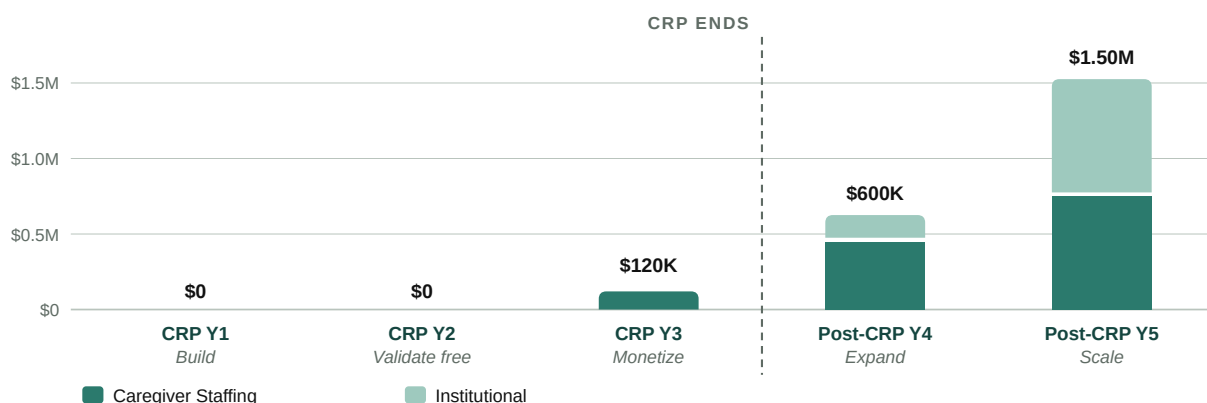


Figure 12. Revenue is intentionally back-loaded: Year 1 builds the system, Year 2 validates Staffing free, Year 3 introduces real billing, and post-CRP expansion adds both Staffing markets and institutional contracts.

How the projection should be read. These projections are not top-down estimates of market share. They are bottom-up scenarios derived from the number of successful caregiver hires Olera can produce in each county and the number of evidence-gated institutional contracts the company can secure. The base case deliberately holds Staffing throughput at 10 hires per month per county even as markets mature, so it excludes the expected 10 to 30 hire mature range. Institutional revenue begins only after CRP evidence exists. The principal assumptions, which are Staffing throughput, price, repeat purchasing, contribution margin, and institutional contracting value, are therefore the same variables the CRP and subsequent buyer negotiations are designed to replace with measured commercial data.

Commercial hiring, workforce expansion, and family care establishment are tracked as related but distinct outcomes. Staffing revenue is earned on successful provider hires; workforce impact is measured through unique entrants, retention, and provider capacity; and CareNavigator-linked family impact is measured by whether cases encountering a documented workforce barrier subsequently establish care. Downstream utilization remains a longitudinal hypothesis until supported by evidence.

Staffing and capital as revenue grows. During the CRP, engineering, research, and market-validation personnel remain central. As paid Staffing expands, Olera adds centralized market operations, worker acquisition, provider success, and sales capacity rather than recreating a full team in every county. As institutional contracts emerge, business development, implementation, analytics, and account-management capacity grow in parallel. Engineering grows more slowly because the platform, portals, and workflows are designed for self-service and automation. Under the illustrative base case, commercial revenue reaches approximately \$120,000 in CRP Year 3, \$600,000 in post-CRP Year 4, and \$1.5 million in Year 5. This revenue meaningfully reduces the post-CRP financing requirement, but is not assumed to eliminate it immediately. The Finance Plan therefore sizes post-CRP capital against the remaining operating gap and the additional capital required to replicate validated Staffing markets and develop institutional contracts, rather than assuming that revenue from the limited CRP markets immediately replaces federal support.

References supporting quantitative benchmarks.

1. Home Care Association of America. Activated Insights Benchmarking Report Now Available. 2025. Reports median professional-caregiver turnover of approximately 75% in 2024.

2. Activated Insights. Caregiver Retention Software for Home-Based Care. Reports recruiting and training costs of up to approximately \$2,700 per replacement caregiver.
3. Activated Insights. 2025 Benchmarking Report for Home-Based Care. Home-care workforce recruitment and retention benchmarking.
4. Centers for Medicare & Medicaid Services. GUIDE Model Frequently Asked Questions. Describes per-patient-per-month dementia care-management payments for coordination and caregiver education and support.
5. Centers for Medicare & Medicaid Services. Guiding an Improved Dementia Experience (GUIDE) Model. Describes care coordination, caregiver support, and alternative payment methodology.
6. Centers for Medicare & Medicaid Services. 2026 Medicare Accountable Care Organization Initiatives Participation Highlights. Reports 511 Medicare Shared Savings Program ACOs serving 12.6 million Traditional Medicare beneficiaries in 2026.
7. U.S. Bureau of Labor Statistics. Home Health and Personal Care Aides. Occupational Outlook Handbook, 2025 to 2035 employment projections. Approximately 760,500 projected openings annually.
8. Agency for Healthcare Research and Quality. Evidence Map on Home- and Community-Based Services and Person-Centered Care for Older Adults. Technical Brief No. 49. Summarizes a 13-state Medicaid HCBS study reporting emergency department use of 52% versus 34% and hospital or rehabilitation stays of 36% versus 24% among participants with versus without unmet HCBS needs.

Modeling note. *Olera-specific throughput, pricing, market counts, timing, and institutional contract values are planning assumptions or CRP hypotheses and are not represented as published benchmarks. Successful hires are commercial events; unique workforce entrants, net capacity added, and care establishment are measured separately.*